Oncology · All disease sites · All lead groups · Any phase · All registry statuses · Hospital: Camden Clark Medical Center, Parkersburg

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Breast Cancer

A012103

Phase III · ALLIANCE · Recruiting · NCT05812807

OptimICE-pCR: De-Escalation of Therapy in Early-Stage TNBC Patients who Achieve pCR After Neoadjuvant Chemotherapy with Checkpoint Inhibitor Therapy

Arm 1: Pembrolizumab x 27 weeks Physician choice: 200 mg IV every 3 weeks, or 400 mg IV every 6 weeks x 4 doses then 200 mg x 1 dose. Subcutaneous option: 395 mg every 3 weeks or 790 mg every 6 weeks. CCTG sites may use 2 mg/kg IV (max 200 mg) every 3 weeks.

Arm 2: Observation x 27 weeks No protocol treatment. Visits at baseline, 12 weeks, and 27 weeks after registration.

Primary endpoint. Recurrence-free survival, non-inferiority of observation. Assumed 3-year RFS 94% with pembrolizumab; observation non-inferior if 3-year RFS is 91% or higher.

A012303

Phase III · ALLIANCE · Recruiting · NCT06876714

ShortStop-HER2: 12 Months vs. 6 Months of HER2-targeted Medications for People With HER2+ Breast Cancer Who Had a Pathologic Complete Response After Chemotherapy Plus Trastuzumab

Arm 1: 12 months (standard) 51 weeks total of trastuzumab-containing therapy (17 cycles of 21 days).

Arm 2: 6 months 27 weeks total of trastuzumab-containing therapy (9 cycles of 21 days).

Primary endpoint. Recurrence-free survival, non-inferiority of 6 vs 12 months. HR margin 1.63, assumed 3-year RFS 95% in the control arm; 1,524 patients, 113 RFS events.

NRG-BR009 (OFSET)

Phase III · NRG · Recruiting · NCT05879926

A Phase III Adjuvant Trial Evaluating the Addition of Adjuvant Chemotherapy to Ovarian Function Suppression plus Endocrine Therapy in Premenopausal Patients with pN0-1, ER-Positive/HER2-Negative Breast Cancer and an Oncotype Recurrence Score </= 25 (OFSET)

Arm 1 Ovarian function suppression (GnRH agonist of investigator's choice, or bilateral oophorectomy) plus aromatase inhibitor for 5 years.

Arm 2 Adjuvant chemotherapy of investigator's choice (NCCN- or ESMO-listed regimen), then ovarian function suppression plus aromatase inhibitor for 5 years.

Primary endpoint. Invasive breast cancer-free survival.

Breast Cancer

S1501

Phase III · SWOG · Recruiting · NCT03418961

Prospective Observational Cohort Study of Patients with Metastatic HER-2+ Breast Cancer at Risk of Cardiac Toxicity

S2212 (SCARLET)

Phase III · SWOG · Recruiting · NCT05929768

Shorter Anthracycline-Free Chemo Immunotherapy Adapted to Pathological Response in Early Triple Negative Breast Cancer (SCARLET), A Randomized Phase III Study

Arm 1: Carboplatin/paclitaxel/pembrolizumab then AC/pembrolizumab Cycles 1-4 (21 days): paclitaxel 80 mg/m2 days 1, 8, 15 plus carboplatin AUC 1.5 weekly or AUC 5 day 1, plus pembrolizumab 200 mg every 3 weeks. Cycles 5-8: doxorubicin 60 mg/m2 + cyclophosphamide 600 mg/m2 every 21 or 14 days, pembrolizumab every 3 weeks (8 neoadjuvant doses)

Arm 2: Carboplatin/docetaxel/pembrolizumab (anthracycline-free) Docetaxel 75 mg/m2 + carboplatin AUC 6 + pembrolizumab 200 mg day 1 every 21 days x 6 cycles with required growth factor

Primary endpoint. Breast cancer event-free survival (non-inferiority of Arm 2)

Female Reproductive System Cancer

NRG-CC008

NA · NRG · Recruiting · NCT04251052

A Non-Randomized Prospective Clinical Trial Comparing the Non-Inferiority of Salpingectomy to Salpingo-Oophorectomy to Reduce the Risk of Ovarian Cancer Among BRCA1 Carriers (SOROCk)

Primary endpoint. Time to incident high-grade serous ovarian, primary peritoneal, or fallopian tube carcinoma (non-inferiority of BLS to BSO).

NRG-GY035

Phase III · NRG · Recruiting · NCT07198074

A Randomized Phase III Trial of Carboplatin, Paclitaxel, Pembrolizumab versus Carboplatin, Paclitaxel, Bevacizumab versus Carboplatin, Paclitaxel, Pembrolizumab, Bevacizumab in the Treatment of pMMR, TP53 Mutated Advanced or Recurrent Endometrial Cancer

Arm 1 (reference) Paclitaxel, carboplatin, pembrolizumab 200 mg every 3 weeks, then pembrolizumab 400 mg every 6 weeks for 14 additional cycles.

Arm 2 Paclitaxel, carboplatin, bevacizumab (or anti-VEGF biosimilar) 15 mg/kg every 3 weeks, then bevacizumab 15 mg/kg every 3 weeks for 28 additional doses.

Arm 3 Paclitaxel, carboplatin, pembrolizumab 200 mg, and bevacizumab 15 mg/kg every 3 weeks, then pembrolizumab 400 mg every 6 weeks for 14 cycles plus bevacizumab 15 mg/kg every 3 weeks for 28 doses.

Primary endpoint. Progression-free survival, each experimental arm vs Arm 1; interim futility and efficacy analysis at 60% information. PFS not reported until accrual completes.

NRG-GY036 (HRD+)

Phase III · NRG · Recruiting · NCT06580314

A Phase III Trial of One vs Two Years of Maintenance Olaparib, with or without Bevacizumab, in Patients with BRCA1/2 Mutated or Homologous Recombination Deficient (HRD+) Ovarian Cancer Following Response to First Line Platinum-Based Chemotherapy

Arm 1 (reference) Olaparib 300 mg PO twice daily for 2 years, with or without bevacizumab 15 mg/kg IV every 21 days (bevacizumab stops at 1 year).

Arm 2 Olaparib 300 mg PO twice daily for 1 year, with or without bevacizumab 15 mg/kg IV every 21 days for up to 1 year.

Primary endpoint. Investigator-assessed PFS by RECIST 1.1 (non-inferiority of 1 year), analyzed as PFS360: modified ITT population of patients on study at least 360 days after randomization, with time at risk starting at day 360.

Female Reproductive System Cancer

NRG-GY037

Phase III · NRG · Recruiting · NCT07061977

A Phase III Study of Induction Pembrolizumab and Chemotherapy Followed by Chemoradiation and Pembrolizumab vs Chemoradiation and Pembrolizumab Both Followed by Pembrolizumab for High Risk Locally Advanced Cervical Cancer

Gastrointestinal Cancer

EA2192

Phase II · ECOG-ACRIN · Recruiting · NCT04858334

APOLLO: A Randomized Phase II Double-Blind Study of Olaparib versus Placebo Following Curative Intent Therapy in Patients with Resected Pancreatic Cancer and a Pathogenic BRCA1, BRCA2, or PALB2 Mutation

Arm A Olaparib 300 mg PO BID, Days 1-28, 28-day cycles x 12 cycles

Arm B Placebo 300 mg PO BID, Days 1-28, 28-day cycles x 12 cycles

Primary endpoint. Relapse-free survival. Follow-up for survival 10 years from Step 1 randomization.

NRG-CC005 (FORTE)

NA · NRG · Recruiting · NCT05080673

Five or Ten Year Colonoscopy for 1-2 Non-Advanced Adenomatous Polyps (FORTE)

Arm 1 Surveillance colonoscopy at 5 years and 10 years after the qualifying colonoscopy (each +/- 1 year).

Arm 2 Surveillance colonoscopy at 10 years after the qualifying colonoscopy (+/- 1 year).

Primary endpoint. Colorectal cancer incidence in years 4 through 11 (non-inferiority of the 10-year-only schedule).

NRG-GI008 (CIRCULATE-US)

Phase II/III · NRG · Recruiting · NCT05174169

Colon Adjuvant Chemotherapy Based on Evaluation of Residual Disease (CIRCULATE-NORTH AMERICA)

Arm 1 Immediate adjuvant chemotherapy: mFOLFOX6 for 3-6 months (6-12 cycles) or CAPOX for 3 months (4 cycles).

Arm 2 Serial ctDNA surveillance every 3 months through year 1, then every 6 months through year 3. Conversion to ctDNA positive allows transfer to Cohort B with a second randomization to Arm 3 or 4.

Arm 3 mFOLFOX6 (12 cycles) or CAPOX (8 cycles) for 6 months.

Arm 4 mFOLFIRINOX for 6 months (12 cycles): oxaliplatin 85 mg/m2, leucovorin 400 mg/m2, irinotecan 150 mg/m2, 5-FU 2400 mg/m2 over 46-48 hours every 2 weeks, no 5-FU bolus.

Primary endpoints. Cohort A: phase II time to ctDNA positivity (or recurrence), phase III DFS. Cohort B: DFS (phase II and III).

Genitourinary Cancers

A032201 (STRIKE)

Phase III · ALLIANCE · Recruiting · NCT06661720

Short TeRm Intensified Pembrolizumab (KEytruda) and Tivozanib for High-Risk Renal Cell Carcinoma - STRIKE

Arm 1: Pembrolizumab 400 mg IV every 6 weeks for up to 8 doses (preferred), or 200 mg IV every 3 weeks for up to 16 doses, per institutional practice.

Arm 2: Pembrolizumab plus tivozanib Pembrolizumab 400 mg IV every 6 weeks for up to 8 doses, plus tivozanib 1.34 mg PO daily Days 1-21 of each 28-day period for up to 24 weeks (cycles 1-2).

Primary endpoint. Investigator-assessed disease-free survival. Assumed 24-month DFS 77% with pembrolizumab, target HR 0.67 (84%); target accrual 1,040.

NRG-GU015 (ARCHER)

Phase III · NRG · Recruiting · NCT07097142

The Phase III Adaptive Radiation and Chemotherapy for Muscle Invasive Bladder Cancer Trial (ARCHER)

Arm 1 4 weeks of radiosensitizing chemotherapy plus hypofractionated radiotherapy, 55 Gy in 20 fractions.

Arm 2 4 weeks of radiosensitizing chemotherapy plus SBRT, 32.5 Gy in 5 fractions.

S2427 (BRIGHT)

Phase II · SWOG · Recruiting · NCT07061964

Single Arm Phase II Study of Bladder Preservation with Immunoradiotherapy After a Clinically Meaningful Response to Neoadjuvant Therapy in Patients with Muscle Invasive Bladder Cancer (BRIGHT)

Head and Neck Cancer

A222301 (HOPE)

Phase III · ALLIANCE · Recruiting · NCT06992427

HIGH-DOSE PROPHYLACTIC GABAPENTIN (HOPE) TO PREVENT OPIOID USE FOR ORAL MUCOSITIS PAIN DURING HEAD AND NECK CHEMORADIOTHERAPY: A PHASE III CLINICAL TRIAL

Hematologic Cancer

EAA173

Phase III · ECOG-ACRIN · Recruiting · NCT03937635

Daratumumab to Enhance Therapeutic Effectiveness of Revlimid in Smoldering Myeloma

Arm A: DRd Daratumumab-hyaluronidase 1800 mg/30,000 units SC (or daratumumab 16 mg/kg IV if SC not tolerated) days 1, 8, 15, 22 cycles 1-2, days 1 and 15 cycles 3-6, day 1 cycles 7-24; lenalidomide 25 mg PO days 1-21 cycles 1-24 (10 mg if CrCl 30-59 ml/min); dexamethasone 40 mg PO days 1, 8, 15, 22 cycles 1-6, then 20 mg cycles 7-12.

Arm B: Rd Lenalidomide 25 mg PO days 1-21 cycles 1-24 (10 mg if CrCl 30-59 ml/min); dexamethasone 40 mg PO days 1, 8, 15, 22 cycles 1-6, then 20 mg cycles 7-12.

Primary endpoint. Overall survival.

Lung, Mediastinal, and Pleural Cancer

A081801

Phase III · ALLIANCE · Recruiting · NCT04267848

Integration of Immunotherapy into Adjuvant Therapy for Resected NSCLC: ALCHEMIST CHEMO-IO

Arm B (sequential) Platinum doublet x 4 cycles, +/- PORT, then pembrolizumab 400 mg IV every 6 weeks x 8 doses (16 cycles)

Arm C (concurrent) Platinum doublet x 4 cycles with pembrolizumab 400 mg on day 1 of cycles 1 and 3, +/- PORT, then pembrolizumab 400 mg every 6 weeks x 6 doses (12 cycles), 16 total cycles of pembrolizumab

LUNGMAP

NA · SWOG · Recruiting · NCT03851445

A Master Protocol to Evaluate Biomarker-Driven Therapies and Immunotherapies in Previously-Treated Non-Small Cell Lung Cancer (Lung-MAP Screening Study)

S1800E (REGN2810)

Phase II/III · SWOG · Recruiting · NCT06616584

LUNGMAP: A Randomized Phase II/III Study of Docetaxel and Ramucirumab with or without Cemiplimab (REGN2810) for Participants Previously Treated with Platinum-based Chemotherapy and Immunotherapy for Stage IV or Recurrent Non-Small Cell Lung Cancer (LungMAP Non-Matched Sub-Study)

Arm A (control) Ramucirumab 10 mg/kg IV Day 1 + docetaxel 75 mg/m2 IV Day 1 (60 mg/m2 allowed) every 21 days, with dexamethasone premedication.

Arm B Ramucirumab + docetaxel as in Arm A + cemiplimab 350 mg IV Day 1 every 21 days. Cycle 1 safety run-in with weekly toxicity checks.

Primary endpoint. Overall survival (phase III), with a phase II interim futility analysis on PFS at 99 events. Accrual goal 378.

Male Reproductive System Cancer; Genitourinary Cancers

NRG-GU013 (High Five)

Phase III · NRG · Recruiting · NCT05946213

THE PHASE III HIGH FIVE TRIAL FIVE FRACTION RADIATION FOR HIGH-RISK PROSTATE CANCER

Arm 1 Ultrahypofractionated SBRT in 5 fractions.

Arm 2 Conventional or moderate hypofractionation in 20 to 45 fractions.

Primary endpoint. Metastasis-free survival on conventional imaging.

Miscellaneous and Metastatic Cancer

A151804

NA · ALLIANCE · Recruiting · NCT04242095

Establishment of a National Biorepository to Advance Studies of Immune-Related Adverse Events

A211901

Phase III · ALLIANCE · Recruiting · NCT05008848

Reaching Rural Cancer Survivors Who Smoke Using Test-Based Cessation Interventions

Intervention 8-week text-based Scheduled Gradual Reduction (Mosio system) with target quit date 8 weeks after enrollment, plus NCI Smokefree.TXT from 2 weeks before to 4 weeks after the quit date (12 weeks of intervention).

Control NCI Clearing the Air cessation booklet, with a quit date set within 8 weeks.

Primary endpoint. Biochemically validated smoking abstinence 6 months after quit date (urine cotinine < 50 ng/mL, or anabasine < 2 ng/mL for those vaping or using NRT).